جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : مينا سمير عبيد قديس
المحافظة : الجيزه **الحي / المدينة :** امبابه **العنوان :** ---
رقم البطاقة : 29204082103655 **رقم التليفون :** 01274864990 **تاريخ تسجيل الإستمارة :** 18-10-2026
العمر(تاريخ الميلاد) : 08-04-1992 **المهنة :** حاصل علي دبلوم

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : اورام ليمفاويه
نتائج الأبحاث : (مرفق)
التشخيص النهائي : اورام ليمفاويه
خطة العلاج : أورام كيماوي علاج تدعيمي وتلطيفي [6396] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: مينا سمير عبيد قديس
National ID: 29204082103655

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

A patient of lymphoma for Supportive and palliative care with follow-up

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



DMS
كيماوى
تعيين
سبع ذارع
الورق على السيستم



National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Name:	MINA SAMIR EBID	**Date:**	23/06/2026
Referred by:	Dr. Rafat Atta	**Age:**	34 years

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

FINDINGS:

(I) Chest:

- Both lungs are clear showing no parenchymal nodules, cavitary lesions, consolidative patches or bronchiectatic changes.
- Multiple enlarged right hilar, cardiophrenic, subcarinal, pretracheal and right paratracheal largest measures 3.7x2.9 cm and as well as enlarged right supra-clavicular, posterior cervical and right lower jugular largest measures 4.7x2.5 cm in maximum axial dimensions.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

(II) Abdomen and pelvis:

- Mild mural thickness of the cecum and ascending colon.
- Multiple enlarged celiac, mesenteric, para-aortic, aortocaval and portahepatis LNs largest measures 1.6x3.7 cm in max axial dimensions.
- Anterior abdominal wall epigastric subcutaneous well defined cystic lesion with wall calcifications measures 15 mm in diameter.
- The liver is enlarged in size showing homogenous CT density with no focal lesions.
- Patent and homogenously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Enlarged spleen with no focal lesion. Hilar splenule measures 22 mm in diameter.
- Normal CT appearance of the pancreas, suprarenal glands, both kidneys, aorta and IVC.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.
- No ascites.

Name: مينا سمير عبيد قديس **MR. :** 12455	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : | **تاريخ وساعة الخروج :**

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 34-year-old male with a history of bariatric surgery 14 years prior and chronic NSAID abuse, which has led to a significant hemoglobin drop to 4 g/dL and repeated blood transfusions for upper gastrointestinal bleeding. Endoscopic evaluation revealed a gastric stricture with ulcerations; biopsy demonstrated chronic gastritis with mildly positive *H. pylori* and esophageal candidiasis (G4). A CT scan of the chest, abdomen, and pelvis showed cervical, mediastinal, and abdominal lymphadenopathy. Complete blood count revealed anemia and thrombocytopenia with a platelet count of 90 × 10⁹/L. The plan was to complete a one-week course of fluconazole and initiate ABVD chemotherapy.

The following day, a PET-CT performed on 9-8-2026 confirmed stage 3B Hodgkin lymphoma with both supra- and infra-diaphragmatic lymphadenopathy. Laboratory results for the first ABVD cycle were pending. The patient was informed about fertility preservation options.

A pharmacy note recorded the patient's weight of 84 kg, height of 170 cm, and BSA of 2 m². Total bilirubin was 2.11 mg/dL, prompting a 50 % dose reduction of vincristine and doxorubicin in the ABVD regimen.

On 30-8-2026, laboratory values improved with bilirubin decreasing to 1.27 mg/dL. The patient was scheduled to receive the first cycle of ABVD with dose adjustments (duflac controloc 1 × 1 × 14, doliprane 1 g 1 × 2 × 10).

تحريرا في ,

التاريخ : 21-09-2026 **الوقت :**

الطبيب المعالج : hager shahin

يعتمد ,

[[DOCTYPE:CT_SCAN]]
CT Scan

OPINION:

- **Pathological supra and infradiaphragmatic LNs with hepatosplenomegaly as described likely neoplastic ?lymphoma for lab and histopathological correlation.**
- **Mild mural thickness of the cecum and ascending colon**

Dr. Sheriff Abdel-Fatah, M.D

HALA

[[DOCTYPE:IHC]]
Immunohistochemistry

Dr.
Elia Anis Ishak
M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.
Shady Elia Anis
M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : مينا سمير عبيد قديس **Analysis ID :** 1072516

Age : 34 Y : 2 M : 2 D **Gender :** Male **Conf. Code :** 6650

Referred By Prof. Dr. :

Clinical Diagnosis : Right deep cervical lymph node

Nature of Specimen : Block (Tru cut biopsy, labelled as 1072515)

Receiving Date : 2026-07-05 **Patient ID :** 783772

IMMUNOHISTOCHEMISTRY REPORT

Technique :

Sections were prepared from the paraffin block then treated by monoclonal antibodies and detection kit (EnVision FLEX) using DAB as chromogen and hematoxylin as counter-stain,using Omnis machine.
The sections were treated against
-CD 30
Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain
The sections were treated against:
-CD 20 \ CD 3 \ Ki67 \ CD 15

Results :

CD30 showed few POSITIVE large neoplastic cells.
CD15 showed POSITIVE neoplastic cells.
Neoplastic cells are NEGATIVE for both CD20 and CD3.
CD20 showed positive small non neoplastic cells.
CD3 showed many positive non neoplastic cells in the background.
15% of neoplastic lymphoid cells are positive for KI67.

Conclusion :

Block, FINDINGS ARE COMPATIBLE WITH CLASSIC HODGKIN'S LYMPHOMA, MIXED CELLULARITY.

Prof. Dr. Shady Elia Anis ***Prof. Dr. Elia Anis Ishak***

This report is electronically signed , any abrasion or modification would render it void .

DR ELIA ANIS LABS
عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : ٠١٢١٢٩٤٥١١٥ - ٠١٢٨٧٠٠٥٢١٧ - ٢٥٧٧٣٢٥٣ - ٢٥٧٩٠٩٣٥

[[DOCTYPE:PATHOLOGY_REPORT]]
Pathology Report

Dr.
Elia Anis Ishak
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Dr.
Shady Elia Anis
M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : مينا سمير عبيد قديس **Analysis ID :** 1072515

Age : 34 Y : 2 M : 2 D **Gender :** Male **Conf. Code :** 9178

Referred By Prof. Dr. :

Clinical Diagnosis : Right deep cervical lymph node

Nature of Specimen : Tru cut biopsy

Receiving Date : 2026-07-05 **Patient ID :** 783772

PATHOLOGY REPORT

Gross :

Multiple grayish white tissue cores, longest measured 1 cm, totally submitted.

Microscopic :

Sections examined from the specimen received revealed cores of dense lymphoid infiltrate, formed of small lymphocytes, plasma cells, few eosinophils with few scattered large cells, showing large nuclei, prominent nucleoli and some are binucleated.

Diagnosis :

Right deep cervical lymph node, Tru cut biopsy, MALIGNANT LYMPHOMA.
- IMMUNOPHENOTYPING IN SEPARATE REPORT.

Prof. Dr. Shady Elia Anis ***Prof. Dr. Elia Anis Ishak***

This report is electronically signed , any abrasion or modification would render it void .

DR ELIA ANIS LABS
عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : ٠١٢١٢٩٤٥١١٥ - ٠١٢٨٧٠٠٥٢١٧ - ٢٥٧٧٣٢٥٣ - ٢٥٧٩٠٩٣٥

[[DOCTYPE:PATHOLOGY_REPORT]]
Pathology Report

Dr.
ia Abd El-Kareem
MB BCh, M.sc., M.D.,
sultant & Lecturer of Pathology
Faculty of Medicine
airo University (Kasr Al-Ainy)

الدكتورة
داليا عبد الكريم
دكتوراه التحاليل الباثولوجية
استشاري و مدرس التحاليل الباثولوجية
كلية الطب جامعة القاهرة (قصر العيني)

Patient Name: مينا سمير عبيد قيس **Age:** 34 Y

Referring Physician: أ.د. أحمد صبحي **Code:**10F-1744526

Clinical Data: Post bariatric gastric surgery stricture with ulceration

Nature of the specimen: Endoscopic biopsy **Date:** 15\06\2026

PATHOLOGY REPORT

GROSS:

Small biopsy portions. Totally submitted.

MICROSCOPY:

Sections examined from the received biopsy reveal gastric mucosal tissue fragments. The lamina propria shows moderate mononuclear inflammatory cellular infiltrate (corresponding to score 3/6), neutrophils (corresponding to score 2/6), and eosinophils (Eos <15/HPF). Mild number of *H.Pylori* organism (corresponding to score 2/6). No atrophy, intestinal metaplasia, dysplasia, or malignancy in the referred biopsy.

CONCLUSION: Endoscopic biopsy:

CHRONIC GASTRITIS, MODERATE INTENSITY, MILD ACTIVITY, MILDLY POSITIVE FOR *H.PYLORI*

Dalia Abd El-Kareem, M.D.

٧٣ شارع ٩ - المعادي (عمارة بيتزا هت) ت : ٠١٠٠٧٥٩٧٣٩١ / ٠١٢٨٣٠٠٥٩٦٩
info.drdalialab@gmail.com

Mina Samir Obeid Qaddis

2026-09-13

Referred by: Prof.Dr. **no doctor**

Hospital: **Goustave Roussey International Egypt**

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- *Enlarged in size (17cm at Rt MCL) homogenous parenchymal echo pattern with smooth surface.*
- *No hepatic focal lesions or intra hepatic biliary radicle dilatation.*
- *Portal vein is patent and not dilated.*

Gall Bladder: -

- Average distention and capacity with no stones or mud
- *. The wall is not thickened.*
- *CBD is not dilated.*

Spleen: -

enlarged 17cm showing large cystic lesion with internal floating particles measures 11x8cm likely abscess , small splenule near the hilum

- **Kidneys: -**
- *Both are average in size, shape and echogenicity*
- *No stones or dilated pelvicalyceal system*
- *No gross lesion*

Para-aortic and pancreatic area*: show no gross abnormalities.*

No ascites

Urinary Bladder: -

- *Not distended enough however no stones or diverticular out pouching*

Uterus and adnexal; *no abnormalities could be detected*

Opinion:

Mild hepatomegaly

Splenomegaly with cystic lesion likely abscess for correlation with contrast CT study

Dr/Nermeen Ramadan.MD

Ultrasound
[[DOCTYPE:ULTRASOUND]]